Ethiopia's PHC at a Glance
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AN ASSESSMENT USING PHCPI FRAMEWORK

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MINISTRY OF HEALTH-ETHIOPIA

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ETHIOPIAN PHC ASSESSMENT IN BRIEF

Introduction

Primary health care (PHC) is the core component of the Ethiopian health system. The country has transformed its PHC system over more than two decades with the introduction of the flagship Health Extension Program (HEP), accelerated expansion of health facilities, and increased production of the health workforce. The Ministry of Health (MOH), with support from its development partners, has been investing in strengthening the health system for many years. Several studies have been conducted to assess health system capacity, availability of various essential health services, financing, performance of the PHC and equity in PHC services in Ethiopia. Efforts to generate evidence for some aspects of PHC were underway. Nevertheless, up-to-date and comprehensive evidence integrating PHC capacity, financing, performance, and equity is lacking. This comprehensive and integrated assessment is done to generate evidence to inform policies and actions to strengthen the PHC system (financing, capacity, performance, and equity) toward universal health coverage (UHC).

Aim and Objectives

Aim

The aim of this assessment is to understand the PHC system, including its financing, capacity, performance, and equity, and its role towards UHC in Ethiopia.

Objectives

1. To assess the status of ***financing*** of PHC in terms of PHC spending, prioritization of PHC, and sources of PHC spending
2. To assess the ***capacity*** of Ethiopia's PHC system and identify strengths and limitations for the delivery of effective PHC in order to work toward UHC and improved health status.
3. To describe the level of ***performance*** of PHC in terms of access, coverage, effective coverage and quality of essential services for key population groups.
4. To examine the situation of ***equity*** in access, coverage, and outcomes of PHC services by income, education, and residence.

Methods and Materials

The national PHC assessment used various methods for different components that include financing, capacity, performance and equity.

Financing

In the financing component, assessment involved the collection and synthesis of total spending on PHC per capita, prioritization of spending on PHC, and sources of spending on PHC. The main data sources were WHO and National Health accounts.

Capacity

The assessment of capacity, on the other hand, utilized the principles, methods, and tools developed by the PHC Performance Initiative (PHCPI) after they were customized to local contexts. Data and information were collected and synthesized from available national and regional documents and datasets. In addition, key informant interviews were conducted with experts at the MOH, regional health bureaus, and woreda health bureaus. These were synthesized and summarized under each of the 33 measures of PHC assessment. The assessments at the regional (nine regions and two city administrations) and federal levels were conducted in three stages:

External assessment: A team of trained evaluation experts from the MERQ Consultancy has assessed the PHC capacity at national and regional levels. The synthesized document prepared from the desk review, qualitative interviews, and quantitative data was used in the evaluation process. Each process was concluded by producing reports, including assessment scores and detailed narrative reports. The teams used the Delphi technique to reach agreement among its members.

Internal/self-assessment: After the external assessment was completed, teams of experts from each region and the federal MOH assessed their PHC capacity using the customized PHC assessment tool. This took 4–5 days in total. The teams from the MOH and regional health bureaus included directors/department heads and at least one expert from each directorate/department. The team from the federal MOH had four participants, who had good knowledge about the country's PHC, from development partners. The overall internal assessment was facilitated by the external assessors from the MERQ Institute.

Validation: One-day validation workshops, which involved members of the external and internal assessment team, were conducted in each region and at the federal MOH. The objectives of the validation workshops were to resolve any differences between the internal and external assessment scores and to enrich the evidence for the scoring. The internal and external assessment teams discussed the scores and reached consensus on the final scores.

Performance

The performance component examined three key dimensions of service delivery, including access to service, quality of care, and service coverage. The analysis was completely based on national and international databases, review of policy documents and national assessments, and review of routine official reports. The major data sources were consecutive demographic and health surveys, the service availability and readiness assessment, global burden of diseases databases, and routine reports on health management information systems.

Equity

The equity domain measured health equity in three dimensions: access, which assessed the difference in perceived financial barriers to care between the highest and lowest wealth quintiles; coverage, which indicated the difference in effective coverage of maternal and child health care services based on the mother's level of education; and outcomes, which highlighted the difference in mortality of children residing in urban or rural areas. In the assessment, we used national and international databases, review of policy documents, and national assessments. The data sources in this assessment were demographic and health surveys, service availability, and the readiness assessment.

Findings

Financing:

According to the WHO estimates, the total PHC spending per capita in Ethiopia was US$ 20 in 2018. It is estimated that 82% of the overall health spending goes to financing PHC. From the government's total health spending, PHC constitutes about 80%. On the other hand, estimates of per capita spending and the share of health spending on PHC based on the recent national health account are US $22.19 and 61%, respectively. The Ethiopian PHC has various sources of funding including development partners, the government and out-of-pocket spending, and the government covers 23% of total PHC spending.

As summarized in figure 1, the Ethiopian national per capita PHC spending and the share of overall health spending on PHC as a percentage of current health expenditure had generally an increasing trend according to the consecutive national health account studies. The government domestic spending on PHC as % of total spending on PHC had declining trend and the trend has changed its direction starting 2010/11

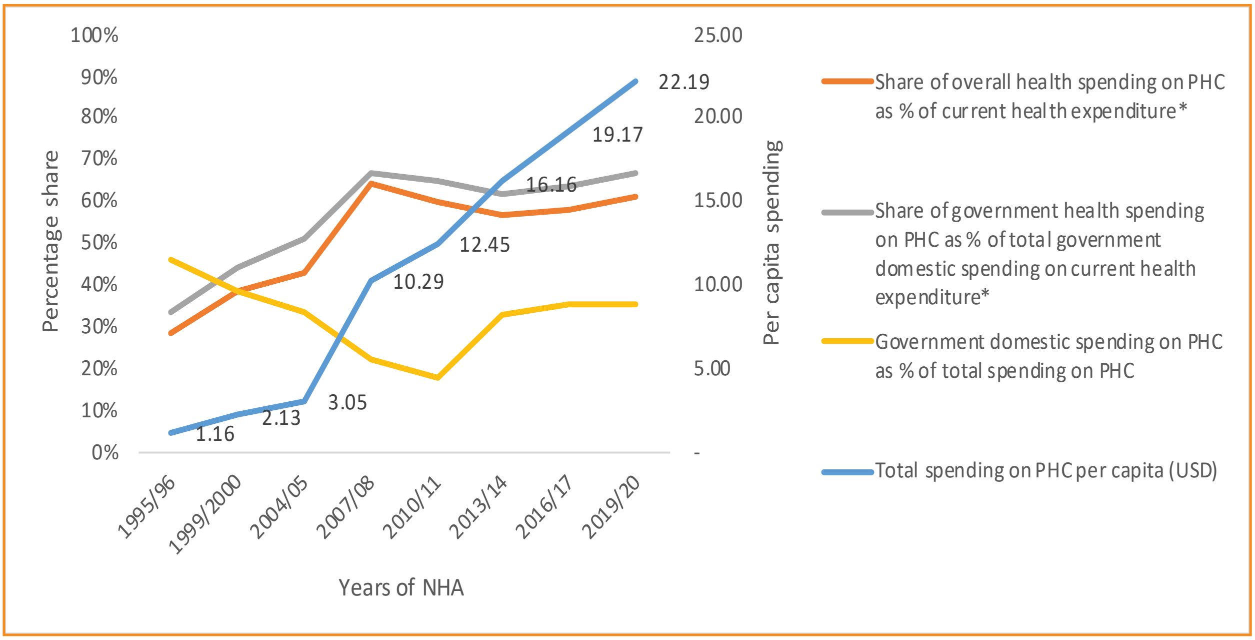

Figure 1: Trends of the financing indicators of Ethiopian primary healthcare.

Capacity:

Governance: With a domain score of 2.7 at the federal level and an average score of 2.5 at the regional level, the governance of PHC in Ethiopia is characterized by the availability of important policy and strategy, social accountability, and quality management documents at the national level. There is also dedicated leadership for PHC at the federal MOH and regional health bureaus. However, the contextualized implementation of national-level policies at regional levels and the strengthening of PHC leadership at subregional levels need further attention.

Inputs: With a domain score of 2.3 at the federal level and an average score of 2.7 at the regional level, this domain is characterized by a complex picture of the availability, distribution, and quality of inputs. At the national level, health facilities, excluding health posts, have, on average, 7 out of the 24 essential medicines, 4 out of the 6 tracer items of basic equipment, 39% mean availability of tracer items for amenities, 42% mean availability for standard precautions for infection prevention, and limited availability of diagnostic tests. Due to the rapid expansion of health facilities, access to PHC has potentially increased from 50.7% in 2000 to more than 90% in 2019. There is a 57% facility-based notification and 18–20% community-based notification of births and deaths. There is promising progress toward the minimum threshold of health-professionals-to-population ratio of 2.3 per 1,000 population, the 2025 benchmark set by the WHO for Sub-Saharan Africa. However, critical gaps in health workforce distribution and quality remain to be addressed. This domain of PHC is also characterized by recent health financing reforms in revenue retention and utilization that have improved the availability of funds for health infrastructure and supplies, the utilization of electronic (IBEX) financial management systems to manage and track budgets allocated from the government, and some variations in the predictability and timing of overtime payments for health workers across regions.

Population Health and Facility Management: With a domain score of 2 at the federal level and an average score of 2.1 at the regional level, key aspects of this domain include a local priority setting that emphasizes a "community first" approach; community engagement through Women Development Armies; a progressively developing community-based health information system and community-based health insurance systems; campaign-based community outreach; a "family health team" at the community level, a health care financing strategy that outlines the legal framework for facility management; and annual reviews of health system performance that emphasize one report, along with hierarchical, supportive supervision.

Performance:

Access: Available number of health facilities is not adequate to meet the population need. The proportion of CBHI membership of eligible households remains low causing significant amounts of out-of-pocket expenditure for health service utilization. In 2016, 61% of women had problems in accessing health care due to cost of treatment while 58 % of women reported distance as a barrier to their health care seeking.

Quality: The quality of services provided at PHC in Ethiopia received a 47% quality composite score. Most quality indicators measuring comprehensiveness, provider competence, and safety were below the 50% line. The two exceptions were TB treatment success rate, which is an indicator of continuity of care, received score of 95%, and provision of information about the diagnosis of their sick children to caregivers, which is an indicator of person centeredness, received scored 92%.

Service coverage: Service coverage was assessed in terms of infectious diseases, non-communicable diseases, and reproductive, maternal, neonatal, and child health services coverage throughout the country. The index score for service coverage was found to be 53%.

- ***Infectious disease:*** The percentage of under-five children with diarrhea who received ORS has increased from 13.1% in 2000 to 30% in 2016. The national tuberculosis case detection and treatment success in the year 2017 was 61% and 77% of people living with HIV in 2020/21 were on anti-retroviral treatment with significant variability across regions (it ranges from 37% in Afar to 94% in Addis Ababa)
- ***Non-communicable diseases:*** According to the 2017 UHC global monitoring report, the prevalence of hypertension among those greater than 18 years of age is 21%.
- ***Reproductive, Maternal, Newborn and Child health:*** Coverage of different services vary across regions of Ethiopia. In 2019, 61% of children in Ethiopian received three doses of DPT vaccine and this coverage ranges from 26.2% in Somalia to 93% in Addis Ababa. The proportion of under 5 children with pneumonia symptoms who sought treatment from health facility increased from 15.8 % in 2000 to 31% in 2016. Using the 2016 DHS data the Effective Coverage (EC) of ANC visits at the population level was 7.5% and national-level EC of institutional delivery was 9.1%. EC for PNC mothers within two days and EC for PNC for newborns was 3.6% and 1.6%, respectively.

Equity:

A comparison of economic barriers to care among different wealth quintile households indicated that there is about a 36% difference favoring the highest wealth quintile. Mortality among under 5-years-old children is about 18% higher in rural settings compared to urban areas. There were differences in the use of reproductive, maternal, neonatal, and child health (RMNCH) services according to maternal level of education (figure 2): the RMNCH index for mothers who attended secondary education, or more is higher by 30% than those who did not get any education. The difference was narrow for three RMNCH indicators, namely, demand satisfied for family planning, children who received ORS and children with pneumonia symptoms taken to health facility. For example, the 2016 DHS data showed that 23.09% of children from uneducated mothers and 39.76% of children from secondary-level-and-above-educated mothers who had pneumonia-like symptoms were taken to a health facility.

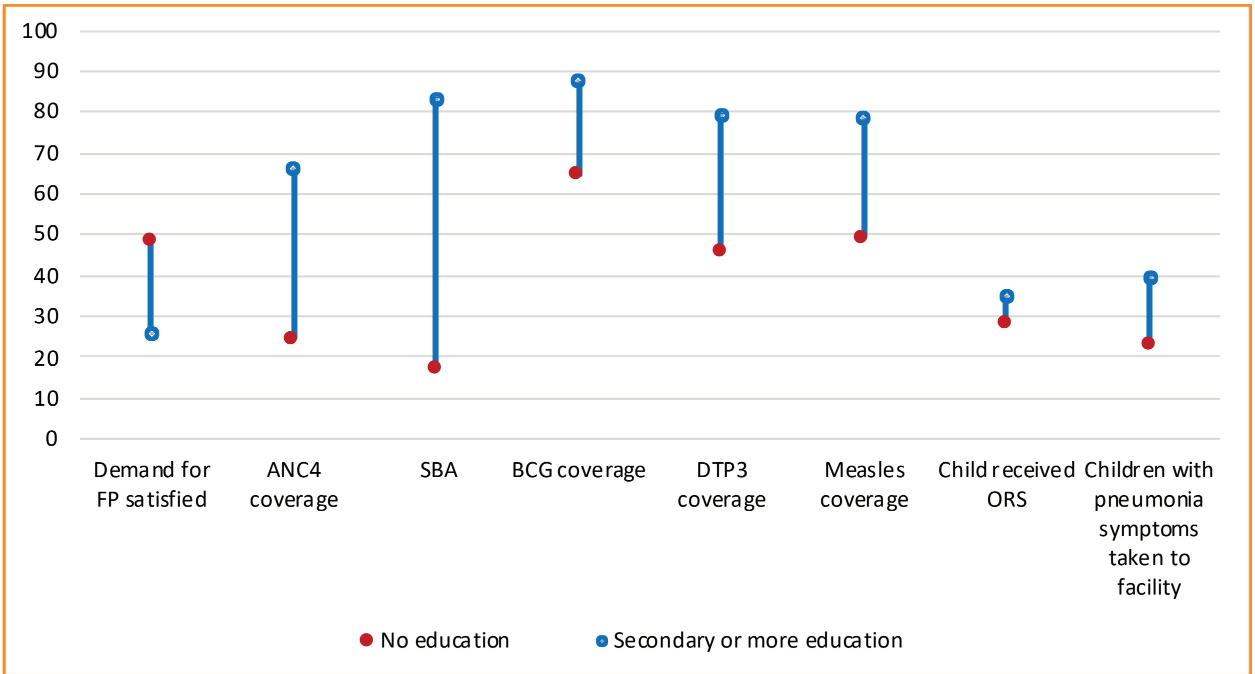

Figure 2: The effect of mothers' level of education on various RMNCH indicators using DHS 2016

Conclusion

This first assessment of the PHC system in Ethiopia using the PHCPI model has revealed the key strengths and weaknesses of the PHC system in the country. The following are the key conclusions of the assessment:

- Based on the WHO's estimates, the total PHC spending per capita is US$ 20. A significant proportion of the overall health spending and government health expenditure goes to finance PHC.
- There are relevant PHC policies and leadership structures at federal and regional levels. However, the capacity to effectively implement these policies and strategies at those levels is suboptimal. The challenges related to major inputs include limited availability of resources, delayed and interrupted supply of supplies, maldistribution of human resources across PHC facilities, and lack of capacity for maintenance of equipment.
- Generally, the majority of women in Ethiopia have problems accessing health care due to financial barriers and geographic hardships. Performance in terms of the quality index is below average, and the service coverage index is average while the effective coverage is very minimal.
- A comparison of economic barriers to care among different wealth quintile households shows that there is about 36% difference favoring the highest wealth quintile, and RMNCH services show differences based on maternal level of education. In all of the measures, mothers with a better education have higher health status.

Recommendations

- With a promising recent progress of the health management information, attention needs to be given to the further improvement of data quality and utilization of information for local decision-making.
- To improve the efficiency of the management at PHC facilities, there is a need to enhance the capabilities of PHC facility and district health office mangers through the provision of on-the-job training, continuing professional development, and supportive supervision.
- Given the critical role of development partners in supporting PHC in Ethiopia, the MOH and RHBs need to improve coordination of this support and focus on areas with high levels of inequities in the delivery of PHC services.
- In order to overcome disparities in health indicators because individual and household characteristics, strengthening the engagement of the private health sector in PHC delivery through the Public Private Partnership initiative would be useful to improve access to PHC services. There is a need for periodic assessment of PHC capacity to regularly evaluate progress, identify gaps, and develop strategies to further strengthen PHC capacity in order to move toward UHC.

Table 1: The national level capacity of PHC

NATIONAL DASHBOARD SHOWING PHC CAPACITY OF ETHIOPIA

Measures	Scores
GOVERNANCE	2.8
Governance and Leadership	3
Measure 1: Primary health care policies (1/2)	4
Measure 2: Primary health care policies (2/2)	3
Measure 3: Quality management infrastructure	3
Measure 4: Social accountability (1/2)	3
Measure 5: Social accountability (2/2)	2
Adjustment to Population Health Needs	2.7
Measre 6: Surveillance	3
Measure 7: Priority setting	3
Measure 8: Innovation and learning	2
INPUTS	2.3
Drugs and Supplies	1
Measure 9: Availability of essential medicines and consumable commodities	1
Measure 10: Basic equipment	1
Measure 11: Diagnostic supplies	1
Facility Infrastructure	1.7
Measure 12: Facility distribution	3
Measure 13: Facility amenities	1
Measure 14: Standard safety precautions & Equipment	1
Information Systems	2
Measure 15: Civil Registration and Vital Statistics	1
Measure 16: HMIS	3
Measure 17: Personal Care Records	2
Workforce	3
Measure 18: Workforce density and distribution	1
Measure 19: QA of PHC workforce	3
Measure 20. PHC workforce competencies	4
Measure 21: Community health workers	4
Funds	3.7
Measure 22: Facility budgets	4
Measure 23: Financial MIS	4
Measure 24: Remuneration	3

Measures	Scores
POPULATION HEALTH AND FACILITY MANAGEMENT	2
Population Health Management	1.8
Measure 25: Local priority setting	2
Measure 26: Community engagement	2
Measure 27: Empanelment	2
Measure 28: Proactive population outreach	1
Facility Organization and Management	2.2
Measure 29: Team-based care organization	4
Measure 30: Facility management capability and leadership	1
Measure 31: Information system use	1
Measure 32: Performance measurement and management (1/2)	1
Measure 33: Performance measurement and management (2/2) – supportive supervision	4